When the MRI laboratory pays a kickback to the doctor, as is common in India, medical malpractice arises. Prices of healthcare tend to skyrocket when individuals are unable to engage in comparison shopping for healthcare services with predictability of expenditures.

In a simple for-profit setting, the incentive of the doctor is not to heal the patient. The incentive of the doctor is to extract maximal revenue. Alongside this, it is efficient to make the patient feel subjectively better. This is done by over-prescribing antibiotics, by prescribing steroids (that briefly make a person feel better), by having a good bedside manner, by earning likes on social media, etc. Such strategies do not heal, but they generate more referrals. In a simple for-profit setting, there is no incentive for the doctor to guide the patient into pathways that prevent future requirements of healthcare.

Public policy initiatives are required to address these market failures in healthcare.

The Indian journey

Under colonial rule, there was an emphasis on the public goods of sanitation and communicable disease. A government committee, led by Sir Joseph Bhore from 1943 to 1946, envisaged a shift in focus from public health to healthcare, and advocated a government-run healthcare system. This was an extremely influential report, and shaped a large reorientation of health policy in the following decades. [5]

By the 1980s, it was clear that these modifications were working poorly. The disease burden was high, owing to weaknesses in public health. The government-dominated healthcare system was working poorly. [6] Individuals were increasingly resorting to private healthcare. Politicians became increasingly aware of the unhappiness of individuals.

Ideally, these difficulties should have generated a fresh focus upon public health. However, building capabilities in public health is difficult. To have tens of thousands of health workers, all over India, who fight mosquitoes, is a difficult problem in public administration.

Health policy in recent decades has instead taken the path of least resistance—to use public money to buy insurance (often from private health insurance companies) for individuals who would obtain healthcare services from private healthcare providers. [7] This is an inefficient path in three ways: Weak public health gives a high disease burden, there is market failure in